



Modified Essay Questions

SCORING KEY

INSTRUCTIONS:

- Please use pencil ONLY, preferably 2B
- Do not fold or bend
- Erase mistakes fully
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- Completely fill in the oval

Please MARK LIKE THIS ONLY:

0 1 2 3 4

Modified Essay 6

Each question within this modified essay question will be marked by a different examiner. The examiner marking one question will not have access to your answers to the other two questions. Therefore, please ensure that you address each question separately and specifically. For example, answer question 6.4 fully, even if you believe that you have partly covered its content in your answers to questions 6.1, 6.2 and 6.3.

You are the consultant psychiatrist who is asked to see Adora, a 26-year-old Filipina woman who emigrated 5 years ago. She is married to Jeff and they have a 4-year-old son, Ben. Jeff works on an off-shore oil rig and is away from home two weeks each month.

Adora has few friends and no family here for support. She often finds it difficult to manage Ben on her own. Ben has been having difficulties getting to sleep.

There is conflict between Adora and Jeff over various things when he is home.

Adora was brought to hospital after she had been observed to be having a fit at her local supermarket at lunchtime. The emergency department registrar believes Adora is experiencing 'pseudo seizures' and refers her to psychiatry for assessment and management.

Question 6.1

You are the consultant who is called by your registrar to discuss the case after their assessment.

Outline (list and justify) the salient features of the history you would like your registrar to focus on. (12 marks)

HISTORY

A.	DETAILED DESCRIPTION OF PRESENTATION • What were the features of the 'fit'. • Previous history of seizure disorder. • Basis for referrer concluding history of pseudo-seizures. • Characteristics of past episodes: injuries, incontinence, aura, postictal recovery precipitants; investigation/treatment; secondary gain issues.	0 1 2 3 4
B.	HISTORY OF PAST AND CURRENT PSYCHIATRIC SYMPTOMS • Depression and/or; anxiety symptoms. • Neurovegetative disturbance. • Alcohol and/or substance use.	0 1 2
C.	HISTORY OF PAST AND CURRENT MEDICATION For general medical conditions; • Anticonvulsants. • Psychotropics. • Antidepressants. • Anxiolytics. • Tranquillizers. • Over the counter preparations. • Aids for sleep, energy.	0 1 2
D.	CLARIFICATION OF CURRENT STRESSORS • Mother/child relationship. • Psychological/behavioural problems in son. • Current care arrangements for child.	0 1 2
E.	CLARIFICATION OF MARITAL RELATIONSHIP • Development of relationship and state of current relationship. • Difficulties with parenting their child. • Past and current conflicts and/or violence.	0 1 2
F.	CLARIFICATION OF CURRENT SOCIAL CIRCUMSTANCES • Social supports including family, friends, local community housing and financial situation. • Difficulties around cultural issues and language differences.	0 1 2
G.	Spare (only to be used after approval from Co-Chairs, Writtens Sub-Committee)	1
H.	Did not attempt	0
I.	Writing	0
Note to Examiner: Please mark all bubbles even if the total adds up to more than 12.		
Note to Pearsons: Please set the maximum mark to 12.		
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Question 6.2

Outline (list and justify) the salient features of the examination you would like your registrar to focus on. (4 marks)

EXAMINATION

A.	PERTINENT POSITIVE AND NEGATIVE FINDINGS FROM THE PHYSICAL EXAMINATION • Evidence for injuries from 'fit'. • Evidence for bruising from possible domestic violence. • Signs for alcohol, substance abuse.	0 1 2
B.	MENTAL STATE EXAMINATION • No evidence for delirium, post-ictal confusion. • Pertinent positive and negative findings with regard to psychopathology. • Observations from the interview on interaction style.	0 1 2
C.	CONTEXTUAL OBSERVATIONS • Information from emergency department staff on behaviour. • Impressions on concerns (for child) and coping style (responding to a crisis). • Report relevant findings from investigations performed in emergency department.	0 1 2
D.	Spare (only to be used after approval from Co-Chairs, Writtens Sub-Committee)	1
E.	Did not attempt	0
F.	Writing	0

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Note to Pearsons: Please set the maximum mark to 4.

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Question 6.3

Outline (list and expand) the framework of the formulation you would like your registrar to present. (4 marks)

FORMULATION

A.	SUCCINCT SUMMARY OF FINDINGS • Reason for assessment. • Relevant findings for basis of diagnosis. • Conclusion and options for immediate management.	0 1 2
B.	CONSIDERATION HAD BEEN GIVEN TO SAFETY AND RISK ISSUES • With regard to mode of presentation. • With regard to Adora's personal safety, and the safety and welfare of her child.	0 1 2
C.	CONSIDERATION HAD BEEN GIVEN TO CONTEXTUAL ISSUES • Source of supports, (husband, family, community) if discharge from emergency department anticipated, versus indication for further admission. • Well-being of child, review of reported sleep difficulties. • Counselling with husband, review of reported conflict issues.	0 1 2
D.	Spare (only to be used after approval from Co-Chairs, Writtens Sub-Committee)	1
E.	Did not attempt	0
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There is conflict between Adora and Jeff over various things when he is home.

Adora was brought to hospital after she had been observed to be having a fit at her local supermarket at lunchtime. The emergency department registrar believes Adora is experiencing 'pseudo seizures' and refers her to psychiatry for assessment and management.

The registrar anxiously informs you that Adora is extremely distressed. The registrar believes a short admission is warranted. However, the social worker is not able to organise acute care for Ben and Jeff has rung from the oil rig, angry, saying that Adora should be at home minding Ben and she is 'just attention seeking'.

The Emergency Department is insisting that she be discharged from their care within the next hour.

Question 6.4

Outline (list and explain) how you would support your registrar to manage this situation. (4 marks)

A.	FACE-TO-FACE WITH REGISTRAR • Express support for registrar. • Acknowledge anxiety provoking situation. • Pressure from emergency department to make a quick decision. • Offer to see patient with registrar. • Review risk issues with registrar.	0 1 2
B.	ON BEHALF OF REGISTRAR • Liaise with emergency department consultant/team. • Defuse the situation. • Negotiate adequate time for safe management.	0 1 2
C.	ANTICIPATING PROBLEMS • Discussion with registrar about how to engage with the husband; understanding the husband's perspective, obtaining collateral history, clarify the husband's views about "attention seeking" • Clarifying potential for conflict of interest with involving husband in decision making. • Identifying further stressors for Adora (for hypothetical scenarios). • Identifying further concerns about welfare for Ben (for hypothetical scenarios).	0 1 2
D.	CONSIDER MANAGEMENT OPTIONS • Offer next day follow up. • Negotiate medical bed. • Explore child care options, either formal services or family availability. • Facilitate home visit from Acute Care team.	0 1 2
E.	Spare (only to be used after approval from Co-Chairs, Writtens Sub-Committee)	1
F.	Did not attempt	0
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